

Clinical Documentation Sample Set

Patient Name: [NAME]

Date of Birth: [DATE]

NHS Number: [NHS-NO]

Address: [ADDRESS], Bradford, West Yorkshire [POSTCODE]

Phone: [PHONE]Email: [EMAIL]

Hospital ID: BRD-442918

Date of Visit: [DATE]

Chief Complaint:

Persistent chest discomfort and shortness of breath for 3 days.

History of Present Illness:

Patient reports intermittent substernal chest tightness associated with mild exertion. Denies reports

occasional dizziness. Past medical history includes hypertension and hyperlipidaemia. Current include

Atorvastatin 20mg daily and Ramipril 5mg daily.

Assessment:

Possible unstable angina. ECG showed nonspecific ST changes. Troponin pending.

Plan:

Admit for observation, [SERIAL-NO], cardiology review, and stress testing.

Consultant: [PROVIDER-NAME], [REG-NO]

Referral Letter

To: Neurology Department, St. Anne's Medical Centre

Referring GP: [PROVIDER-NAME]

Practice Address: [ADDRESS] Surgery, Leeds [POSTCODE]

Patient: [NAME]

Date of Birth: [DATE]

NHS Number: [NHS-NO]

Insurance [INS-ID]

Dear Colleague,

I am referring Ms. Emily Carter for evaluation of chronic migraine headaches with increasing over the past

six months. She reports associated photophobia, nausea, and visual aura. MRI brain imaging pe [DATE] was unremarkable.

The patient currently works as a financial analyst and reports significant work-related stres medications

include Sumatriptan 50mg PRN and Propranolol 40mg twice daily.

Please assess and advise regarding long-term migraine management and further investigation if

Kind regards,

[PROVIDER-NAME]

[REG-NO]

Additional Clinical Note (Page 2)

Field

Value

Patient Name

Rebecca Collins

DOB

[DATE]

NHS Number

[NHS-NO]

Home Address

[ADDRESS], Manchester [POSTCODE]

Mobile

[PHONE]Emergency Contact

Daniel Collins - [PHONE]Employer

Northern Telecom Services Ltd.

Progress Note:

Patient presented for follow-up regarding poorly controlled Type 2 Diabetes Mellitus. Recent 9.2%. Patient admits inconsistent adherence to dietary recommendations. Complains of intermittent numbness feet.

Lab Results:

Fasting glucose: 14.1 mmol/L

Creatinine: 92 umol/L

eGFR: 78 mL/min

Treatment Plan:

Increase Metformin to 1g BID and initiate diabetic foot care referral. Follow-up in 8 weeks.

Clinician Signature: [PROVIDER-NAME]